

Crisis Counseling Final Paper

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Introduction

In this paper, I will examine the situation that happened to me in real-life, that is, my first psychotic episode. My first psychotic episode was a truly life changing moment and needed to be handled very carefully. My behavior needed to be evaluated to be a sign of illness and not just a normal feeling, and I needed to be taken to the mental hospital.

I will analyze and discuss how I would apply Howard Stone's A-B-C Model of crisis intervention to the selected crisis situation. This paper will contain three main sections for A, B, and C.

A: Achieving Contact with the Person in Crisis

When I was having my psychotic episode - my first one - I felt happy. I felt elated. I felt bliss. It was diagnosed as mania - a kind of high and happy feeling. I was using the computer as usual when all of a sudden, I felt a great burden lifted off of my mind, and I felt that I could see color and happiness in the world for the first time.

I took a picture of a flower and posted a rosy poetic-type of Facebook Post. People liked that post - but I wonder if people wondered why I was so happy and blissful.

I was so blissful that I got in my car and drove to my therapist - to tell her the good news and happiness that I felt. You see, my therapist already had 10 sessions with me, and evaluated me as having problems that need to be solved with simply coping mechanisms. I started therapy because I had social anxiety, and I left the last session as a suicidal, depressed person.

My therapist knew something was wrong with me, but didn't know the true root problem that I had - she simply treated the symptom of anxiety and depression with a support group and time off from school.

When I met my therapist, I was exuberant. I thought that there were cameras everywhere that were recording me because I had a delusion of grandeur. My therapist gave me a piece of paper to read, and I couldn't read it. This was a clear and objective sign that something was wrong with my psychology.

One thing that I can evaluate that the therapist did a good job on was that when I first met her, she was calm and accepting. This allowed me to open up and express my exuberant happiness. Also, having both of us sit down so that we can both be calm is a good step for A in the Steps.

My therapist would always ask, "How are you doing? Is there anything new?" And she would always sit facing me, but would have me facing a little toward the right. I personally think that this is not a good method because we should be face-to-face and honest and open to each other. By having her face me directly and with me facing a different direction, causes her to observe me closely without allowing me to observe her back. To take it to the extreme, I know that in torture, the torturer tries to blind the person so that he cannot observe or know what is going on. Thus, I believe that the therapist did wrong in this aspect.

However, another perspective can be that by allowing me to face toward the wall and slightly to the right of the therapist, I am able to ramble and talk about everything about myself and "talk too much." By facing a wall, I have no inhibitions and nothing stops me from talking - I just keep talking and get everything off my chest. This may be a tricky way for the therapist to get all the information that she needs.

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A transcript of the process can be seen below:

Dennis (thinks to himself): “Wow! I finally see color and beauty in this world! I feel a great pressure being lifted off from my head! I want to take a picture of a flower and post it on Facebook and share my happiness with the world! For the first time I feel happy!”

Dennis heads to the garden and takes a photo of a sunflower - so beautiful. He posts it on Facebook.

Dennis (thinks to himself): “My only friend is my therapist - I want to tell my therapist that I’m doing so well and I feel great!”

Dennis first calls his therapist to make an appointment: “Hi Therapist! I feel so happy. I feel extremely happy! I feel like all my problems have been solved and a great burden has been lifted off of me!”

Therapist: “Wow, Dennis, that sounds nice, but I’m a little concerned. I think it’s unusual to be that happy, and thus, I would like to meet with you to see you in-person. Is everything okay?”

Dennis: “Yes, everything is okay. I’ll be there in 40 minutes.”

Dennis grabs his skateboard and gets in his car and drives 40 minutes to his therapist.

Dennis enters the room, where the therapist stands up and greets him.

Therapist: “Hi Dennis, it’s good to see you. How are you feeling? Have a seat.”

Dennis sits in the chair and begins to explain his situation.

B: Boiling Down the Problem to Its Essentials

The key concept of B - in the ABC method - is to listen, and have the client talk, and to respond in a reasonable fashion, and to hear the client respond again. The details of the chapter in Stone’s book is about how to do these steps in the best way possible.

For example, one must listen for a long time to hear all the details of the story and let the client express and release all their emotions. Then, it's important to respond with love, and empathy, rather than correction or rebuke. We have to be accepting of all sinners, regardless of their sin or experiences. Then, the client will respond with acceptance and explain the real problem.

I would like to propose that with my personal situation, the therapist did a really good job in terms of Concept B.

To continue the previous section, I would be slightly faced to the right and I explained the reason for me coming here. I said that I felt happy for the first time, and I explained my odd and delusional thinking. For example, I would explain that I think the building is filled with monitoring equipment to analyze my every word and movement. Did the therapist respond at this point? No, she did not - and I would like to point out that this is a great move, according to Stone, because she was intent on hearing the entire story, not just affirming me that I did good so far.

In fact, she was sitting at the edge of her seat, writing things down, and giving me eye contact. Although Stone may not explicitly state this, but it is important for the therapist to truly care for the subject - not just have behaviors down to some robotic fashion. And I would like to say that the therapist truly cared for me. This is important because when she asks me, the ultimate question at the end of the session, I would respond in a way that is best for everyone, and is true to my heart and is an answer in love.

And just to jump the gun, I want to state what the ultimate question is: "I think you need to go to the mental hospital - Dennis can we go?" And I hear this question all the time whenever

I talk about my problems to a counselor after having schizophrenia for more than 10 years. I just want to emphasize again, that this question must be asked after all listening is done, and has the best interest of everyone in mind. It must be asked sincerely.

But to go back to the timeline of me explaining why I am here, the therapist listened to all explanation of my mental state, and she asked me a question.

“Can you read this piece of paper.”

She hands me a piece of paper and I look at it, but am dumbfounded as to what it says. I am scatter-brained.

“I don’t understand what it means.”

And at that point, she knew that something was not good with my mental state. She evaluated that a person like me, who got into one of the top colleges in the world, should be able to read such a paper easily. She was narrowing down the problem and finding a specific diagnosis. This is the proper approach to Concept B.

Concept B is about boiling down - going to the core problem and simplifying the problem. It starts and ends with the goal of diagnosing the issue so that we can take steps to address the problem. But I understand that when I was talking about my current situation, there was a lot of information.

I would talk about my trip to Korea, how I felt so depressed and suicidal, and the difficult time of navigating the airport as a young, inexperienced youth, the sudden relief I felt that was otherworldly, and my taking a picture of a flower and uploading it to Facebook, and then my sudden desire to share to the world and to my therapist the wonderful feeling that I had.

With all this talk, the therapist listened intently, and wanted to give a test to try to narrow down the issue. I believe the therapist was thinking, “Okay, so there is a lot of information, but is there danger? No. Is there anything odd about his story? The only thing that is odd is that he is very happy - being very happy is no sin, but it is a change, and a change it is. But the thing is, he is too happy. The question I have is: is this mania?”

And with that question, she applied a test - to read this piece of paper.

Looking back, I would say that she made the smartest move in having me read the paper. Right now, I believe that paper was information on mania or schizoaffective disorder, and that I was to read it and see if I match with any of the symptoms or questions.

But the fact that I couldn’t even understand any of the text - that “it looks just like a blob of text,” was a sure sign that my brain was going through some big changes.

She made the evaluation, “Yes - he is having a manic episode right now, and thus, we need to head to the hospital.” It’s probably the most correct, smart, and important, and right move that a person can make. This is because she is a trained therapist, and thus, she knows that it is important to get immediate help. Why? Because the shorter the gap between episode and treatment, the better the prognosis.

I had to say, “Yes.” When she was so sure and adamant to go to the hospital, I knew that I needed to go. In my evaluation as of right now, I am very pleased with the performance of my therapist and am thankful even to this day.

A transcript can be seen below:

Therapist: “So, Dennis, can you explain your situation to me? How did you get so happy?”

Evaluation: according to Stone or others, it may be stated that we should give an open-ended question, and that is what Therapist did when she asked me to explain my situation - that’s a very general question. But the Therapist asked a little bit of a pointed question when she asked how I got so happy.

There are several things to this: the first, is that she is being very direct. She is pointing out a specific thing for examination, and this may take me aback by me thinking, “Why? Is there something wrong with my happiness?” And to be honest, I think I will share in Concept C Section - I loved this happiness and I saw nothing wrong with it. The therapist evaluates mania as an evil that must be killed, but to me, I found happiness to be something so blissful and good - something I got to experience for the first time in my life. Thus, there is a tension: the structure and Leviathan of medical science and therapy, versus the client, who knows the symptoms and problems not as an academic textbook, but real experience in the client’s soul. But as we’ll learn in Concept/Section C, I must die to myself - I must listen to the therapist and their theories or I will be held down and involuntarily injected with medicine.

Dennis: “It all started when I was in Korea with my family. I felt so mellow and depressed. I would walk on the balcony of apartments, and look down and try to jump down. But when I looked at the great height, and the thought of actually dying, I didn’t want to die. I realized that it is a very terrible thing to suicide, and that I wasn’t feeling nearly as bad as to actually suicide. I just had a melancholy feeling and a desire to wade in my sadness.”

Therapist: “So, why did you feel so sad?”

Evaluation: I understand it may not be so wise to interrupt the client in asking a probing question, but I think the Therapist did a good job. It is very important to find the reason for the sadness to make a diagnosis and decide on what action to take next. If there is a very terrible reason for the sadness, we must know; and thus, this question must be asked, and it was good of the Therapist to ask.

Dennis: “Well, you see, I had to leave temporarily from my college in order to rest and recover. You see, I failed my classes and it was too hard for me. And you see, academics was everything for me in high school, and so to fail in college makes me lose all hope and meaning in life.”

Therapist: “Can you continue the story?”

Dennis: “Well, my time in Korea was a melancholy experience, and I eventually flew back to my home in California. And it is this point at which something very unique and strange happened.”

Therapist begins to get ready to write everything down that’s said next.

Dennis: “When I sat down, I felt a great burden lifted from me, and I felt happy for the first time. It was a great blissful happiness - something I’ve always wanted and never felt in my life.”

C: Coping Actively with the Problem

This is probably the saddest and toughest part of this paper. The reason is because the doctor had to go against my will.

What happened was, the Therapist walked with me to the Mental Hospital. They put me in the Emergency Room and observed my behavior to see if I need to go in-patient.

I had been in the ER before because I was suicidal in the past. In the past, I was extremely scared and didn't know the process. But this time, I was very happy - extremely happy.

I would laugh and joke and run around. The doctors knew this was a mania.

Evaluation: the diagnosis of me being manic was a precise and easy conclusion to make. This allowed them to find the right way to cope with this behavior and apply the right healing procedure, which we'll see (at least the next step), was for me to become in-patient.

They brought me to the mental hospital as an in-patient, and I would laugh so hard that I could hardly breathe. The truth was, I loved this mania. I didn't want them to take it away. But they said that we need to stop the mania as soon as possible to allow for a good prognosis. If we do not give him the maximum dosage of Zyprexa, he will never be able to function as well as before.

Evaluation: I believe that the doctor meant well. But we will see that there will be a conflict between the doctor and me - between years of research and the emotions and will of a maniac.

And so the doctor talked to me firmly, "Take this medicine." And I knew their motive - to stamp down on this mania and make me normal.

So I refused the medication.

Then, they said that if I don't take this medicine, I will be strapped down and forced to take the medicine.

Evaluation: the doctors were firm - they knew, from experience and from research, that the best thing for me, and for everyone, was for me to take the medicine. But I would not want to take this medication - this doesn't matter - fact is fact, and we must follow the Leviathan.

I took the medicine, and I became a zombie - that's common with Zyprexa.

I followed the rules and was eventually released from the hospital.

I lost my will and I became a zombie.

As soon as I got out of the hospital, I tried to wean off the medication as soon as possible.

"They are trying to get rid of my happiness by making me a zombie! I must get off this medicine!" And I eventually was free of medicine and was happy. I loved this new version of myself.

I eventually had a relapse, and after many visits to the mental hospital, I became compliant on my medication, and to this day, I still believe the taking medication is the right thing to do, but that we should have freedom - to try different medication and find the right one. But I also must say, the freedom to take medication or not is a God-given right that society must give.

Evaluation: in the end, research and doctors' experience won out - today, I am taking my medication as necessary. But as a young 20 year old, I was so afraid of taking medication and had the emotional thinking that this medication is making me a zombie and I'm losing that happy mania. And so we need to allow for a man to choose his will, even if it may be bad, according to research. Because the will and freedom is essential to true happiness. If I stayed as a zombie, I would be so melancholy and unhappy with my life. And so, to relate this to our class, I must say

that following the books is good, but for Concept/Section C, we must deal with patients as people who have a will and as a person, as well.

Conclusion

Laid out in this paper, is the dramatic experience that I felt in my first psychotic episode. It was a bliss of happiness - a mania. And I felt alive for the first time. But the therapist and doctors evaluated this according to the DSM-4 and all the medical literature and procedures, and decided to take my will away and give me medicine. It was frightening. And it is this issue: doctor's wisdom vs client's will, that makes a big issue in this paper.

The doctor and therapist accurately diagnosed according to their knowledge of psychology and medicine and they used the law to enforce their thinking. There is no answer that I want to give in this paper, except to illustrate that Crisis Counseling may not have a perfect answer for every scenario.

The therapist was a trainee and wasn't fully licensed, and the doctor was experienced and best in his field for schizophrenia. And I was a human being, who had fear and desires.

But I think the conclusion of the matter is that the counselor and the client must seek for the best way forward. If the client resists, it causes problems. We need to speak logically and clearly on equal terms, and give hope and grace and love - the love of Christ.

I think it is important to note that in retrospect, I truly am thankful for everyone involved in my crisis. The therapist was incredibly kind in meeting me unexpectedly, and diagnosed correctly. There were a lot of things that I was saying, with a lot of emotion (unduly happiness), but she boiled the situation down to a point - "He's having mania."

With the kind contact (Concept A), the diagnosis (Concept B), and the firm command to take medication (Concept C), I was able to be in a safe place while having my mania and come to this point in life, where I am thankful for being alive and follow every doctor's order gladly.

Works Cited

Howard W. Stone, *Crisis Counseling*, 3rd ed. (Minneapolis: Fortress Press, 2009).

Scott Floyd, *Crisis Counseling: A Guide for Pastors and Professionals* (Kregel Academic & Professional, 2008), 280.